

INFORMED CONSENT / REFUSAL: OSELTAMIVIR (TAMIFLU®)

Patient Name: _____

Personal Health Number (PHN): _____

Date of Birth: _____

Facility/Care Home: _____

1. BACKGROUND

You (or the resident you represent) are being offered Oseltamivir (Tamiflu®). In British Columbia Long-Term Care facilities, this medication is frequently administered via "standing order" during influenza outbreaks for two purposes:

1. **Treatment:** To treat a resident who already has symptoms of influenza.
2. **Prophylaxis (Prevention):** To prevent infection in residents exposed to the virus, even if they have no symptoms.

2. EVIDENCE SUMMARY (BENEFITS & RISKS)

Based on systematic reviews of clinical trial data (Cochrane Collaboration, BMJ).

POTENTIAL BENEFITS

- **Symptom Duration:** If taken within 48 hours of symptom onset, Oseltamivir may shorten the duration of influenza symptoms by approximately **16 to 24 hours** (roughly one day) [1, 2].
- **Hospitalization & Complications:** High-quality systematic reviews have found **insufficient evidence** to conclude that Oseltamivir significantly reduces the risk of being admitted to hospital, developing pneumonia, or developing other serious complications [1, 3].
- **Prevention (Prophylaxis):** When taken to prevent flu during an outbreak, Oseltamivir can reduce the risk of developing symptomatic influenza. However, a large number of patients must be treated to prevent one case of symptomatic flu.

POTENTIAL RISKS & SIDE EFFECTS

- **Nausea:** Occurs in approximately 1 in 28 patients treated [1, 4].
- **Vomiting:** Occurs in approximately 1 in 22 patients treated [1, 4].
- **Headache & Psychiatric Effects:** Headaches and rare neuropsychiatric events (confusion, delirium) have been reported [1].

- **Renal Consideration:** Dosage must be adjusted for patients with reduced kidney function (common in the elderly) to avoid accumulation of the drug [5].

3. DECISION

Please select ONE of the following options:

[] OPTION A: I CONSENT to the administration of Oseltamivir.

I understand the potential benefits and risks outlined above. I authorize the administration of Oseltamivir for Treatment (if symptomatic) or Prophylaxis (if recommended by the Medical Health Officer during an outbreak).

[] OPTION B: I DECLINE the administration of Oseltamivir.

I have reviewed the evidence regarding the marginal benefits and potential side effects (nausea/vomiting). I **do not** consent to the administration of this medication for either treatment or prophylaxis. I understand that standard supportive care (fluids, fever management, comfort measures) will still be provided.

Name of Substitute Decision Maker (if applicable): _____

Relationship to Resident: _____

Signature: _____ **Date:** _____

Witness Name/Signature: _____

REFERENCES

1. **Jefferson T, Jones MA, Doshi P, et al.** Neuraminidase inhibitors for preventing and treating influenza in healthy adults and children. *Cochrane Database of Systematic Reviews*. 2014;(4):CD008965. (Identified that Tamiflu shortens symptoms by approx. 16.8 hours in adults but found no clear evidence of reduction in hospitalizations or pneumonia).
2. **Ebell MH, Call M, Shinholser J.** Effectiveness of oseltamivir in adults: a meta-analysis of published and unpublished clinical trials. *Family Practice*. 2013;30(2):125-133. (Found a reduction in symptom duration of approx. 21 hours).
3. **Dobson J, Whitley RJ, Pocock S, et al.** Oseltamivir treatment for influenza in adults: a meta-analysis of randomised controlled trials. *The Lancet*. 2015;385(9979):1729-1737. (Noted increased risk of nausea and vomiting; NNH ~22-28).
4. **The NNT Group.** Neuraminidase Inhibitors for Treatment of Influenza. *TheNNT.com*. Accessed 2024. (Summarizes Cochrane data: NNH for vomiting is 22; NNH for nausea is 28).
5. **BC Centre for Disease Control.** Influenza Control: Guidelines for Long-Term Care Facilities. (Notes requirement for renal dosing adjustments).